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Tips for Choosing a Pediatric Formula

“Should I switch my baby’s formula? She has X, Y, and Z symptoms.”

This is one of the most common questions in Pediatrics, whether in-person or virtual. **Bottom line:** Only a very small percentage of children are medically indicated for a switch in formula.

*Breastfeeding should be the primary option, but these are formula options to consider when needed.

The two most common medical indications for switching formula are true GERD or cow milk protein allergy.

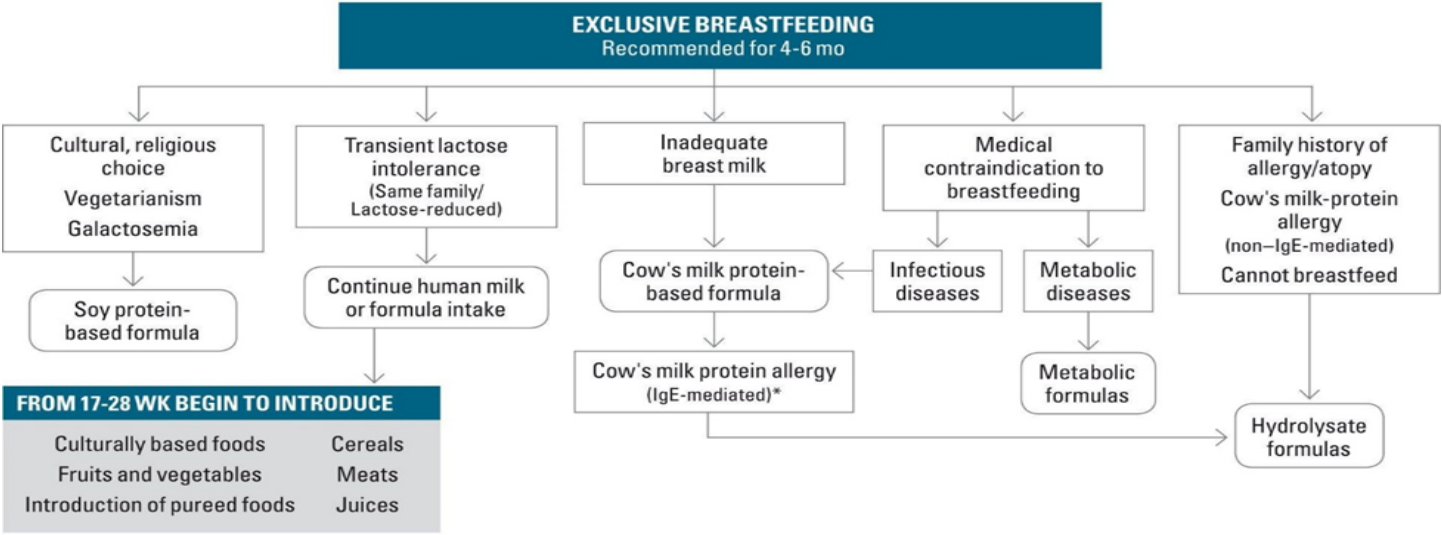
- **GERD:** For formula-fed infants, consider extensively hydrolyzed formulas in addition to usual routine measures (keep upright after feeding for 20 mins, avoid overfeeding, etc.)
- Cow milk protein allergy:
 - Most common food allergy in young children; more prevalent in infants.
 - Often presents within days or weeks of introducing cow milk-based formula, although may occur in exclusively breastfed infants from maternal diet transmission.
 - Various presentations: Acutely with vomiting, diarrhea, lethargy. Or insidious onset of blood-streaked, mucousy, loose stools in otherwise well-appearing children.

Diarrhea and colic are generally temporary and unlikely caused by formula.

- For most infants with transient diarrhea or colic, switching formula is generally not needed, but this topic is controversial.
- It’s often best to wait it out and allow the child’s digestive system to adapt to the original formula, assuming underlying medical issues are ruled out.
- Switching formula is reasonable, but again, controversial.

Formula Types	Brand Name Examples	Notes/Indications
Standard Cow Milk Formula	Enfamil/Enfamil Neuropro Similac Advance Earth’s Best Various Common Generics	Typically contains lactose, but lactose-free versions exist. Most children should be fine with standard cow milk formula.
Partially Hydrolyzed	Enfamil Gentlease Similac Total Comfort Similac 360 Total Care Gerber Good Start	Not shown to significantly prevent atopic disease. Not used for true cow milk allergies. Might be considered for general gassiness or mild reflux, but rarely medically indicated for true disease processes.
Extensively Hydrolyzed (“Hypoallergenic”)	Enfamil Nutramigen Enfamil Pregestimil Similac Alimentum Similac ExpertCare Gerber Extensive	Generally costlier and less palatable. Typically used for true cow milk protein allergy, or strong history of atopy. May reduce incidence of atopic dermatitis, but does not reduce risk of asthma or other allergies. May consider for formula-fed infants with GERD
Amino Acid Based (“Elemental Formula”)	EleCare Neocate Nutramigen AA PurAmino Alfamino (Nestle)	For infants who do not tolerate extensively hydrolyzed formula or symptoms of protein intolerance persist despite trial of hydrolyzed formula. Infants with malabsorption issues
Soy (Also Lactose-Free)	Enfamil ProSobee Similac Soy Isomil Earth’s Best Soy Gerber Good Start Soy	Cultural or religious choice Vegetarianism Galactosemia Hereditary lactase deficiency (very rare) *True lactose intolerance is extremely rare, but can be transient, particularly after bouts of diarrhea. **Generally, not recommend for cow milk allergies due to cross-reactivity.

FIGURE
FEEDING ALGORITHM FOR TERM INFANTS



Abbreviation: IgE, immunoglobulin E. Adapted from Gamble Y, et al.¹ *On a clinical basis, differentiation of IgE-mediated or non-IgE-mediated cow’s milk-protein allergy is difficult and there is a cross-reactivity. Therefore, feeding a protein hydrolysate formula is suggested.

Note. From *How to choose an infant formula* [Table], by Mary Beth Nierengarten, 2021. Contemporary Pediatrics. <https://www.contemporarypediatrics.com/view/how-choose-infant-formula>

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